

WFA Hands-On Activity Guide

Instructor Resource

Wilderness First Aid Course
2-Day Comprehensive Training
Curriculum Year 2026

This guide provides comprehensive hands-on activities, drills, and simulations to supplement classroom instruction. Activities are organized by type and difficulty, with setup requirements, teaching cues, and learning objectives.

SECTION 1: INDIVIDUAL SKILL DRILLS

Timed, Repeated Practice for Core Competencies

Tourniquet Speed Drill

Setup: One tourniquet per student, simulated limb (mannequin or partner's thigh over pants)

Activity: Apply to standard in under 60 seconds. Time it. Repeat until consistent.

Teaching Cue: Real tourniquets need to be applied under 60 sec in field stress

Notes: Common errors: not tight enough, over joints, forgetting time

Glove Don/Doff Drill

Setup: Pairs of nitrile gloves, contaminated object (red-painted sponge)

Activity: Put gloves on, 'treat contaminated patient,' remove gloves without contaminating hands

Debrief: Show contamination with UV powder (optional) or just discuss technique

Teaching Cue: Most providers contaminate themselves during removal

Direct Pressure Endurance Station

Setup: Simulated wound (foam pad wrapped in gauze, red paint), partner lying on ground

Activity: Apply firm direct pressure for the FULL 10 minutes. Students must maintain pressure without releasing.

Notes: Most students are surprised how long 10 minutes feels. Often lift at 1-2 minutes.

BVM (Bag Valve Mask) or Rescue Breathing Station

Setup: CPR mannequin or partner with pocket mask

Activity: Maintain rescue breathing at correct rate (1 breath every 5-6 sec) for 3 minutes

Teaching Cue: Use a metronome or timer. Most students go too fast.

AVPU Assessment 30-Second Drill

Setup: 'Patient' actor who instructor secretly assigns an AVPU level to

Activity: Student assesses and reports AVPU level in under 30 seconds. 5 rotations, different AVPU levels each time

Vital Signs Speed Assessment

Setup: Partner patient lying on ground

Activity: Assess and document pulse rate, resp rate, skin signs, and AVPU in under 3 minutes

Notes: Students often spend 10+ minutes on vitals — this drill emphasizes efficiency

SECTION 2: TEAM SKILLS STATIONS

4-6 Person Teams for Coordination & Communication

Logroll Coordination Station

Setup: Mannequin or volunteer on tarp, 5-person team

Activity: Complete 3 perfect logrolls with in-line stabilization. Count aloud. No movement except on '3.'

Debrief: Swap head provider. Different body types change difficulty.

Notes: Most teams drop alignment on first attempt. Repetition builds coordination.

Improvised Litter Build & Carry

Setup: 2 hiking poles, tarp or emergency space blanket, webbing/rope

Activity: Build a functional litter from scratch in under 10 minutes, load 'patient,' carry 50 meters

Variants: Tarp only (fold into litter), poles + clothing, commercial litter (if available)

Notes: Patient packaging is as important as litter build. Pad bony prominences.

Hypothermia Packaging Drill

Setup: Sleeping bag, space blankets, chemical heat packs, sleeping pad

Activity: Team packages a 'hypothermic patient' correctly and safely in under 8 minutes

Evaluation Criteria: Ground insulation first, vapor barrier, heat packs correctly placed, patient secured, no rough movement

Teaching Cue: Time pressure reveals what students think is most important vs. what actually is

Helicopter LZ Selection & Setup

Setup: An open field/meadow, flags or colored tape, survey equipment (optional)

Activity: Teams select and mark an appropriate LZ, note wind direction, identify approach hazards

Debrief: Compare choices — what made one site better than another?

Optional: Describe or demonstrate proper approach path (never from rear)

Mass Casualty Triage Exercise

Setup: 4-5 actor patients scattered across an area:

Patient 1: Walking, minor cuts (GREEN)

Patient 2: Unconscious, not breathing → starts breathing when airway opened (RED)

Patient 3: Screaming loudly with non-life-threatening fracture (YELLOW/GREEN)

Patient 4: No breathing, no pulse (BLACK)

Patient 5: Controlled hemorrhage, altered LOC (RED)

Activity: 3-person team triages all 5 patients using START triage in under 5 minutes

Notes: The screaming patient always gets over-prioritized. The quiet patient (5) is often missed.

SECTION 3: ASSESSMENT PRACTICE GAMES

Low-Stakes, High-Repetition Learning

SAMPLE History Speed Round

Setup: Pairs

Activity: One student is 'patient' with a prepared backstory card. Other student takes SAMPLE history. Time it. Complete in under 4 minutes.

Rotate: 5 different 'patients' in 20 minutes

OPQRST Pain Relay

Setup: 4 teams of 3. One person knows the OPQRST answer for a patient.

Activity: Team must complete full OPQRST in under 3 minutes, relay correct info to instructor

Teaching Cue: Emphasizes difference between S and O in SAMPLE (subjective report vs. pain quality)

AVPU Bingo

Setup: Laminated bingo cards with clinical findings in squares. Instructor reads scenarios.

Activity: Students mark which square (e.g., 'responds only when you shout their name' = VERBAL)

Teaching Cue: Low-stakes repetition builds pattern recognition

The Mnemonic Race

Activity: Pairs. One student must explain full meaning of each letter in a mnemonic from memory. Other student checks card.

Mnemonics: XABCDE, SAMPLE, OPQRST, DCAP-BTLS, AVPU, AEIOU-TIPS, FAST, MIST

Teaching Cue: Builds automatic recall under stress

SECTION 4: ENVIRONMENTAL SIMULATIONS

Real-World Context for Emergency Response

Low-Light Assessment

Setup: Dim/shade conditions, flashlights only

Activity: Students perform a full primary survey on a 'patient' using only a headlamp

Teaching Cue: Most wilderness emergencies happen at dawn/dusk or in poor weather

Gloved Assessment in Cold

Setup: Winter gloves (thick insulated)

Activity: Students try to assess pulse, apply dressings, and operate equipment while wearing winter gloves

Teaching Cue: Fine motor skills are severely limited in cold. This is humbling.

Communication Drill (No Eye Contact)

Setup: Students assess a 'patient' who is a mannequin — no verbal feedback

Activity: Full XABCDE, SAMPLE, SOAP note with no confirmation from patient

Teaching Cue: Prepares for unconscious patients who cannot respond

Terrain Challenge Carry

Setup: Improvised litter, rough terrain, slopes, obstacles

Activity: Carry a 'patient' 100 meters over uneven terrain as a team

Notes: Reveals the physical demands of wilderness evacuation. Most students are surprised.

SECTION 5: COGNITIVE & DECISION-MAKING ACTIVITIES

Higher-Order Thinking & Clinical Judgment

The 6-Hour Decision Point

Setup: Scenario brief read aloud. Students must document SOAP note and make evacuation decision.

Activity: 'You are 6 hours from trailhead. This patient has X, Y, Z. What do you do? What is your plan? Who goes for help?'

Debrief: Compare decisions. Discuss what changed the decision.

Treatment Timeline Challenge

Setup: Groups of 3. Whiteboard or paper.

Activity: Given a multi-trauma patient, groups have 5 minutes to write their complete treatment sequence in order.

Debrief: Compare sequences. Discuss what's first, what can wait.

Wilderness vs. Urban Comparison

Discussion: 'In an urban emergency, you do X. In wilderness, you do Y — and here's why.'

Topics: Prolonged patient contact, modified treatment protocols, improvisation, evacuation priority

SECTION 6: CAPSTONE ACTIVITIES

Full Integration & Assessment (Day 2)

The Full Patient Encounter (30-40 min)

Setup & Activity:

- One student leads, others assist
- Full XABCDE, SAMPLE, OPQRST, head-to-toe, vitals q15min for the scenario duration
- SOAP note documentation
- Evacuation decision with rationale
- MIST handoff to 'EMS' (instructor)
- Graded with skills assessment form

Skills Station Rotation (Blitz Assessment)

Setup: 8-10 stations, 5 minutes each, one skill per station

Stations: Tourniquet, splinting, rescue breathing, spinal clearance decision, EpiPen, heat stroke cooling, hypothermia packaging, MIST report, FAST assessment

Activity: Students rotate as pairs or individually. Quick feedback at each station.

Document prepared for WFA instructors. Adapt all activities to your course context, student population, and available resources.